DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

Memorandum for Record

Date: June 9, 2025

From: Kevin D. Turnage, MSN Ed, RN, AMB-BC, NHDP-BC, NPD-BC, NE-BC

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Subject: **Summary of Policy Compliance Reviews Post Governance Change**

This memorandum documents the results of policy compliance reviews conducted following the removal of the Policy and Procedure Officer from the policy approval process at Phoenix Indian Medical Center (PIMC) effective May 28, 2025.

A total of 10 policies were reviewed. All 10 policies (100%) failed to meet full regulatory compliance or clinical safety standards as required under CMS Conditions of Participation (42 CFR §482.12), CLIA, HIPAA, EMTALA, and The Joint Commission standards such as LD.01.03.01, MM.01.01.03, and QSA.02.08.01.

Summary of Reviewed Policies and Identified Issues

Policy Title	PolicyStat ID	Compliance Gap / Error
Intravenous Electrolytes – Storage, Preparation, and Administration	18096583	Missing CMS §482.25(b) reference
Intravenous Electrolytes – Storage, Preparation, and Administration	18096583	No AHA ACLS integration
Intravenous Electrolytes – Storage, Preparation, and Administration	18096583	Omitted TJC MM.01.01.03 requirements
Intravenous Electrolytes – Storage, Preparation, and Administration	18096583	No training or QA documentation
Intravenous Electrolytes – Storage, Preparation, and Administration	18096583	No IHM Pharmacy cross-reference

Stroke Alert	18050273	No AHA Get With The Guidelines® (GWTG) references
Stroke Alert	18050273	Lacks NIHSS use and thrombolytic protocols
Stroke Alert	18050273	Missing CMS §482.45 citation
Stroke Alert	18050273	No RQI performance integration
Emergency Contraception	18183654	No ACOG or FDA guidance cited
Emergency Contraception	18183654	Omitted CMS §482.13(b) patient rights
Emergency Contraception	18183654	Lack of pharmacist/provider training requirements
Adverse Drug Reactions	18272415	No FDA MedWatch reporting process
Adverse Drug Reactions	18272415	No IHS I-STAR linkage
Adverse Drug Reactions	18272415	Missing CMS and ISMP guidance
Adverse Drug Reactions	18272415	TJC MM.08.01.01 not cited
Clinical Laboratory Reporting of Infectious Diseases	18164239	No CDC/state timeframe references
Clinical Laboratory Reporting of Infectious Diseases	18164239	Omission of CLIA §493.1109(b) documentation
Clinical Laboratory Reporting of Infectious Diseases	18164239	No urgent vs routine reporting definition
Cytology Screening Performance	18308966	No CMS CLIA or IHS references
Cytology Screening Performance	18308966	Lacks TJC QSA.02.08.01 citation
Cytology Screening Performance	18308966	No defined QA thresholds or retraining standards
Cytology Result Reporting	18239697	No CAP/CLIA documentation standards
Cytology Result Reporting	18239697	Fails to define reporting timeframes
Cytology Result Reporting	18239697	Lacks cross-referencing with laboratory QA policy
Microtomy Priority and Routine Sectioning Standards	18266343	No workload or QA audit provisions

Microtomy Priority and Routine Sectioning Standards	18266343	Lacks CMS CLIA performance tracking language
Labeling of Specimen Containers, Colored Cassettes, & Slides	18265459	No reference to TJC National Patient Safety Goals
Labeling of Specimen Containers, Colored Cassettes, & Slides	18265459	Omitted CAP and CLIA labeling compliance rules
Asbestos Awareness and Control	18233131	Missing OSHA 1910.1001 standard citation
Asbestos Awareness and Control	18233131	No IHS environmental safety or staff education tracking

Legal Reasoning Behind Systemic Failure

The removal of the compliance oversight role from the policy approval process constitutes a structural governance failure. CMS §482.12 requires the hospital's Governing Body to ensure that the hospital policies are current and legally compliant. The Joint Commission mandates under LD.01.03.01 that governance ensures patient safety and adherence to applicable law and regulations. By bypassing expert regulatory review, the Governing Body assumes direct accountability for any non-compliant or unsafe policy implemented post-removal.

The fact that 100% of the reviewed policies failed compliance review confirms a systemic pattern of neglect. In the event of patient harm, improper billing, or regulatory breach, this failure may expose the institution and individual leaders to liability under the False Claims Act, whistleblower protections, or tort law for administrative negligence.

Conclusion

The findings in this memorandum confirm a breakdown in institutional compliance and policy governance at PIMC. Immediate corrective action is necessary to restore structured policy review and fulfill federal and accreditation requirements.

Respectfully Submitted,

Digitally signed by Kevin Turnage
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